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147 x 26.25 mm (Tablet)

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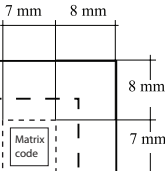
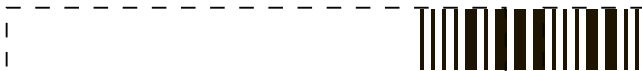
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Product / Item type
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Country : VIETNAM
Plant : Thu Duc
Operator : Nguyen Quoc Bao
Initiated date : DD-MM-YYYY
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Approval of text date: _____ Signature: _____

Final approval date: _____ Signature: _____



Telfast®
Fexofenadine HCL
180mg

1. NAME OF THE MEDICINAL PRODUCT

Telfast 180 mg film-coated tablets

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

Fexofenadine hydrochloride 180.00 mg
For one film-coated tablet.
For the full list of excipients, see section 6.1.

3. PHARMACEUTICAL FORM

Film-coated tablets.

4. CLINICAL PARTICULARS

4.1. Therapeutic indications

Telfast 180 mg is indicated for the relief of symptoms associated with chronic idiopathic urticaria and allergic rhinitis in adults and children 12 years of age and older. Symptoms treated effectively include sneezing, rhinorrhea, itchy nose/palate/ throat, itchy/watery red eyes.

4.2. Posology and method of administration

Posology

Seasonal Allergic Rhinitis and Chronic Idiopathic Urticaria
Adults and Children 12 Years and Older:

The recommended dose of TELFAST tablets is 180 mg once daily with water. A dose of 60 mg once daily is recommended as the starting dose in patients with decreased renal function.

At-risk subjects:

Studies conducted in at-risk adult patients (impaired renal or hepatic function) show that it is not necessary to adjust the posology of fexofenadine hydrochloride in these patients.

Method of administration

Oral use.

4.3. Contraindications

History of hypersensitivity to any of the ingredients.
Children less than 12 years of age due to the lack of safety and efficacy data.

4.4. Special warnings and precautions for use

Patients with cardiovascular disease or a history of cardiovascular disease should be alerted to the fact that antihistamines as a therapeutic group have been associated with the undesirable effects tachycardia and palpitations (see section 4.8).

There are limited data on administration in the elderly and patients with kidney or liver failure. Fexofenadine hydrochloride should be used with caution in these patients (see section 4.2).

Excipient with known effect:

This medicinal product contains less than 1 mmol sodium (23 mg) per tablet, that is to say essentially 'sodium-free'.

KSF/556849

4.5. Interaction with other medicinal products and other forms of interaction

Fexofenadine is not metabolised in the liver and therefore does not interact with other medicinal products via hepatic mechanisms.

Combinations requiring precautions for use

- Topical gastrointestinal agents, antacids and adsorbent agents: Administration of an antacid containing aluminium and magnesium hydroxide salts 15 minutes prior to intake of fexofenadine causes a reduction in bioavailability, probably due to binding in the gastrointestinal tract, and thus reduced gastrointestinal absorption of fexofenadine. An interval (more than 2 hours, if possible) should be allowed between administration of topical gastrointestinal agents and fexofenadine. No drug interactions with omeprazole have been observed.

- The co-administration of fexofenadine with erythromycin or ketoconazole showed an increase (two or three-fold) in plasma levels of fexofenadine. These changes are not accompanied by any effects on QT interval and are not associated with an increase in undesirable effects compared to those observed when the agents are taken separately. Animal studies have shown that the increase in plasma levels of fexofenadine observed after co-administration of erythromycin or ketoconazole appears to be due to an increase in gastrointestinal absorption and a decrease in either biliary excretion or gastrointestinal secretion.

4.6. Fertility, pregnancy and lactation

Pregnancy

Teratogenic Effects: Pregnancy Category C.

As no teratogenic effects have been observed in animals, no malformative effects are expected in humans. To date, substances causing malformations in humans have been shown to be teratogenic in animals during well-conducted studies in two species.

Limited animal studies have not evidenced any direct or indirect harmful effects on gestation, embryo/fetal development, parturition or postnatal development.

No relevant clinical data are currently available to assess the potential teratogenic or fetotoxic effect of fexofenadine when administered during pregnancy.

Therefore, as a precaution, this medicinal product should preferably not be used during pregnancy unless absolutely necessary.

Breast feeding

There is no information on the excretion of fexofenadine in human milk after intake of fexofenadine by breast-feeding women. Nevertheless, diffusion of fexofenadine into human milk has been observed following administration of terfenadine in breast-feeding women.

Administration of fexofenadine is therefore not recommended in breast-feeding women.

4.7. Effects on ability to drive and use machines

Fexofenadine Zentiva 180 mg film-coated tablets have no or negligible influence on the ability to drive and use machines. However, before driving or performing complex tasks, it is advisable to assess individual response so as to identify susceptible subjects who could have an unusual reaction with this medicinal product.

4.8. Undesirable effects

The following adverse reactions have been reported in adults during controlled clinical studies, with a comparable incidence to placebo:

- Nervous system disorders: common (> 1/100, < 1/10): headache, drowsiness, dizziness
- Gastrointestinal disorders: common (> 1/100, < 1/10): nausea not known: dry mouth
- General disorders and administration site conditions: uncommon (> 1/1 000, < 1/100): fatigue

The following adverse reactions have been reported in adults during post-marketing surveillance. The frequency is not known and cannot be estimated from the available data.

- Immune system disorders: Hypersensitivity reactions, including angioedema, chest tightness, dyspnoea, flushing and other systemic anaphylactic reactions have been reported.
- Psychiatric disorders: Insomnia, nervousness, sleep disorders or nightmares/excessive dreaming (paroniria)
- Cardiac disorders: Tachycardia, palpitations.
- Gastrointestinal disorders: Diarrhoea.
- Skin and subcutaneous tissue disorders: Rash, urticaria, pruritus

In placebo-controlled clinical studies in children aged 6 to 11 years, headache was the most frequently reported adverse reaction (1.0%). The incidence of this effect was comparable to that reported in the placebo group.

4.9. Overdose

The symptoms reported during fexofenadine hydrochloride overdose are drowsiness, dizziness, fatigue and dry mouth. Doses up to 60 mg have been administered twice daily for 2 weeks in children, and single doses up to 800 mg and doses up to 690 mg twice daily for 1 month, or 240 mg once daily for a year have been administered to healthy volunteers without causing any clinically significant adverse events compared to placebo. The maximum tolerated dose of fexofenadine hydrochloride has not been established.

Management of massive overdose:

- symptomatic treatment,
- monitoring of vital functions.

Fexofenadine is not dialyzable. There is currently no known antidote.

5. PHARMACOLOGICAL PROPERTIES

5.1. Pharmacodynamic properties

Pharmacotherapeutic group:

Antihistamines for systemic use

(R: Respiratory system), ATC code: R06AX26.

Fexofenadine hydrochloride is a rapid and long-acting antihistamine, with a selective action on peripheral H1 receptors, and devoid of sedating and anticholinergic effects at the recommended doses.

Clinical studies conducted in adults to study the inhibition of histamine-induced wheals and erythema using single and twice daily doses of fexofenadine hydrochloride demonstrated that the antihistaminic effect of the compound starts within one hour, achieving its maximum effect at 6 hours and lasting 24 hours.

There was no evidence of a decrease in these antihistaminic effects after 28 days of administration. A dose-response relationship was found to exist for oral doses ranging from 10 mg to 130 mg. Maximum inhibition in skin wheal and erythema areas exceeded 80%.

No cardiotoxic effects have been evidenced in clinical studies, even at higher than recommended doses.

No significant differences in QT interval were observed in seasonal allergic rhinitis patients given fexofenadine hydrochloride up to 240 mg twice daily for 2 weeks when compared to placebo. Also, no significant change in QT interval was observed in healthy volunteers given fexofenadine hydrochloride up to 60 mg twice daily for 6 months, 400 mg twice daily for 6.5 days and 240 mg once daily for 1 year, when compared to placebo. In children aged 6 to 11 years, no significant difference in QT interval was observed following administration of 60 mg of fexofenadine hydrochloride, twice daily for 2 weeks compared to the placebo group. Fexofenadine at concentrations 32 times greater than the therapeutic concentration in humans had no effect on the delayed rectifier K⁺ channel cloned from human heart.

5.2. Pharmacokinetic properties

Fexofenadine is rapidly absorbed following oral administration, with T_{max} occurring at approximately 1–3 hours post dose. C_{max} is approximately 289 ng/mL following administration of 120 mg as a single daily dose and approximately 494 ng/mL following administration of 180 mg as a single daily dose.

Fexofenadine is 60–70% plasma protein bound.

Fexofenadine does not undergo hepatic metabolism. It is very weakly metabolised since it is the main substance identified in urine and faeces. The plasma concentration profiles of fexofenadine follow a bi-exponential decline with a terminal elimination half-life ranging from 11 to 15 hours after repeated administration. The single and repeated dose pharmacokinetics of fexofenadine are linear for doses up to 120 mg twice daily.

The major route of elimination is via biliary excretion, and 10% of the administered dose is excreted unchanged in the urine.

5.3. Preclinical safety data

450 mg/kg administered to dogs twice daily for 6 months did not show any toxicity other than occasional emesis. Also, in single-dose dog and rodent studies, no treatment-related gross findings were observed following autopsy.

Radiolabelled fexofenadine hydrochloride in tissue distribution studies of the rat indicated that fexofenadine did not cross the blood-brain barrier.

Fexofenadine hydrochloride was found to be non-mutagenic in various in vitro and in vivo mutagenicity tests.

In a reproductive toxicity study in mice, fexofenadine did not impair fertility, was not teratogenic and did not impair pre- or postnatal development.

No carcinogenic potential was demonstrated in studies conducted with terfenadine.

6. PHARMACEUTICAL PARTICULARS

6.1. List of excipients

Microcrystalline cellulose, pregelatinised maize starch, croscarmellose sodium, magnesium stearate, hydroxypropyl methylcellulose (E-15), hydroxypropyl methylcellulose (E-5), povidone, titanium dioxide (E 171), colloidal anhydrous silica, macrogol 400 and pink iron oxide, yellow iron oxide blend.

6.2. Incompatibilities

Not applicable.

6.3. Shelf life

3 years.

6.4. Storage

Do not store above 30°C.

6.5. Nature and contents of container

Box contains 1 blister X 10 film coated tablets
Box contains 2 blister X 10 film coated tablets
Box contains 3 blister X 10 film coated tablets
Box contains 5 blister X 10 film coated tablets
Box contains 100 blister X 10 film coated tablets
Not all pack sizes may be marketed.

7. MANUFACTURED BY

SANOVI VIETNAM SHAREHOLDING COMPANY

Lot I-8-2, D8 Road, Saigon Hi-Tech Park, Long Thanh My Ward, Thu Duc City, Ho Chi Minh City, VIETNAM

8. DATE OF REVISION OF THE TEXT

Jul 2025 (based on French SmPC June 2021)

Opella.